

## HOW TO OPEN A RESIDENTIAL CARE or ASSISTED LIVING FACILITY in Oregon

RCF, ALF & Memory Care — licensed by ODHS, Aging and People with Disabilities.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

### The Oregon model — read this first

Oregon licenses residential care facilities (RCFs) and assisted living facilities (ALFs) as community-based care. The license comes from the Oregon Department of Human Services (ODHS), Aging and People with Disabilities (APD), under OAR Chapter 411, Division 054 and ORS 443.400 to 443.455. Both provide room, board, and personal care and services on a 24-hour basis in a home-like, person-centered setting — the main difference is the building: an ALF offers apartment-style units with a private bathroom and kitchenette, while an RCF offers residential rooms. Either can add a Memory Care Community endorsement (OAR 411-057) for dementia care.

**Two things matter more than anything else when you start.** (1) Your administrator must hold an Oregon Residential Care Facility Administrator (RCFA) license — full-time, on-site at least 40 hours a week. (2) You must have WRITTEN confirmation of licensure from ODHS before a single resident moves in. Build your whole timeline around getting that written confirmation first, and never let a move-in date get ahead of it.

**The three models, in one line.** RCF = residents live in residential rooms — the most flexible and common way to start. ALF = residents live in apartment-style units (lockable lever-handle door, private bathroom, kitchenette) — a more independent model. Memory Care = an endorsement added to an RCF or ALF to serve residents with dementia, with dementia-trained staff and a secure, purpose-built environment.

### How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a facility — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or licensing advice; requirements, fees, and forms change, so verify each step with the agency named (see Part 12) before you rely on it.

### The journey at a glance

Ten milestones take you from idea to an operating Oregon RCF or ALF.

|   |                                 |                                                                                         |
|---|---------------------------------|-----------------------------------------------------------------------------------------|
| 1 | Choose your model               | RCF (rooms), ALF (apartments), and whether to add a memory care endorsement (Part 2)    |
| 2 | Form the business & insure      | Entity, EIN, NPI, insurance (Part 3)                                                    |
| 3 | Secure a licensed administrator | A full-time Oregon RCFA, on-site at least 40 hours a week (Part 5)                      |
| 4 | Plan the building               | Work with ODHS Facilities, Planning & Safety BEFORE you pull a building permit (Part 4) |
| 5 | Write your policies             | Policy manual, resident forms & disclosures built to Division 054 (Parts 6–8)           |
| 6 | Apply to ODHS                   | Submit the SE 0570 application at least 60 days before expected licensure (Part 6)      |

|           |                                                    |                                                                                       |
|-----------|----------------------------------------------------|---------------------------------------------------------------------------------------|
| <b>7</b>  | <b>Prove you can operate</b>                       | Fiscal responsibility, a 12-month pro forma, a nurse, and your staffing plan (Part 6) |
| <b>8</b>  | <b>Screen &amp; train staff</b>                    | Background checks through your QED, caregiver & dementia training (Part 7)            |
| <b>9</b>  | <b>Pass the survey &amp; get WRITTEN licensure</b> | Before any resident moves in (Part 6)                                                 |
| <b>10</b> | <b>Admit residents &amp; grow</b>                  | Serve on a residency agreement & person-centered service plan (Parts 8-11)            |

**Rule of thumb.** *The building is the long pole in the tent — construction, code review, and FPS sign-off take longer than any paperwork, so engage FPS before you pull a permit. And never move a resident in before ODHS's written confirmation of licensure — even one resident, even briefly, is operating without a license.*

## PART 1

# What This Business Is

**In short:** *A licensed community-based care facility — residential rooms (RCF) or apartments (ALF), optionally with memory care — where older adults and people with disabilities receive room, board, personal care, and services 24 hours a day in a home-like setting. Paid privately and by Oregon Medicaid.*

A residential care or assisted living facility gives older adults and people with disabilities a place to live where room, board, personal care, and services are available around the clock — help with bathing, dressing, grooming, and medications; meals and housekeeping; activities and social life; and coordination of health care — in a home-like setting that emphasizes choice, dignity, privacy, and independence. In Oregon this is community-based care, licensed by ODHS, Aging and People with Disabilities.

Be clear-eyed about what kind of business this is. Unlike in-home care, where you can start small and light, a residential care or assisted living facility is a real-estate and operations business first: you're responsible for a building, around-the-clock staffing, meals, and the safety of people who live under your roof. That makes it more capital-intensive and slower to open — but it also builds a durable asset and a steady, occupancy-based revenue stream once you're established.

## The three models, in plain terms

- **Residential care facility (RCF):** residents live in furnished or unfurnished rooms and receive care and services — the most flexible and common way to start, from small homes to larger buildings.
- **Assisted living facility (ALF):** residents live in apartment-style units — each with a lockable lever-handle door, a private bathroom, and a kitchenette — with care and services available; a more independent, apartment model.
- **Memory care community:** an endorsement added to an RCF or ALF to serve residents with Alzheimer's or other dementia, with dementia-trained staff and a secure, purpose-built environment.

**Key terms, in plain English.** RCF = Residential Care Facility (rooms). ALF = Assisted Living Facility (apartments). ODHS = Oregon Dept. of Human Services (licenses you). APD = Aging and People with Disabilities (the ODHS division). Division 054 = the licensing rules (OAR 411-054). FPS = Facilities, Planning & Safety (reviews your building). RCFA = Residential Care Facility Administrator (the license your administrator must hold). SE 0570 = the license application. QED = Qualified Entity Designee (submits your background checks). Person-centered service plan = each resident's individualized care plan. AAA = Area Agency on Aging. NPI = National Provider Identifier.

## Why this business — and why now

Oregon has strong demand for community-based care: a growing older population, a state long-term-care system that deliberately favors home-like settings over institutions, and Medicaid funding that follows residents into licensed facilities. The trade-off is real regulatory structure — a building that meets code, a state-licensed administrator, trained and screened staff, and an ODHS survey — but meeting those standards is exactly what lets families, hospitals, and case managers trust you with a loved one.

**Start with the model that fits your capital.** Many successful operators start with a residential care facility — it keeps your building and capital requirements more manageable while you learn to operate (screening, service planning, staffing, surviving your first survey) on a smaller scale — and expand into apartments or memory care later. Taking on a large purpose-built facility as a first project is the harder path.

## PART 2

# The Oregon License — RCF, ALF & Memory Care

**In short:** *The credential you need is a community-based care license from ODHS under Division 054. You choose the license type that matches your building — and the one rule that trips people up: written licensure before any move-in.*

## 2.1 Choose your license type

- **Residential care facility license:** for a facility built to the residential care building requirements (OAR 411-054-0200) — residential rooms rather than full apartments.
- **Assisted living facility license:** for a facility built to the assisted living building requirements (OAR 411-054-0300) — apartment units, each with a lockable lever-handle door, a private bathroom, and a kitchenette.
- **Memory care community endorsement:** added to either license under OAR 411-057 to advertise and provide dementia care, with dementia-specific staffing, training, environment, and disclosures.

Choosing between an RCF and an ALF is really a decision about the building and the resident experience — rooms in a shared home versus private apartments — and it has cost, timeline, and marketing consequences, so make it deliberately and early. The memory care endorsement is a separate decision layered on top: it reaches a large and growing population but adds staffing, training, environment, and disclosure requirements. Many operators start with one clear model and add the endorsement or a second license later.

***The rule that trips people up — license before move-in.** You may NOT move a single resident into the facility until you have received written confirmation of licensure from the ODHS Community-Based Care Licensing Unit. Opening the doors early — even to one resident, even briefly — is operating without a license. Never let a move-in date get ahead of that written confirmation.*

## PART 3

# Form Your Business & Insure — Step by Step

**In short:** *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

## 3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Oregon you form the LLC through the Oregon Secretary of State, Corporation Division. Oregon also evaluates your ownership and, for Medicaid, discloses owners at the 5% threshold, so your ownership structure matters from the start. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

1. **File your Articles of Organization with the Oregon Secretary of State.** Online at [sos.oregon.gov](https://sos.oregon.gov) (Corporation Division) — the LLC filing fee is about \$100. Name the LLC and list a registered agent (this can be you).
2. **Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank and your ODHS application will want it.
3. **Calendar the annual report.** Oregon LLCs file an annual report (about \$100) each year with the Secretary of State to stay in good standing.
4. **Set up state tax and employer accounts.** Register with the Oregon Department of Revenue and Employment Department for payroll and employer obligations before you hire.

*No newspaper publication in Oregon. Oregon has no LLC publication requirement — forming the entity is a straightforward Secretary of State filing.*

## 3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

1. **Go to [IRS.gov](https://irs.gov) and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
2. **Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
3. **Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
4. **Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

## 3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it when you contract with ODHS to serve Oregon Medicaid residents. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

1. **Create an I&A account.** At [nppes.cms.hhs.gov](https://nppes.cms.hhs.gov), set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
2. **Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

**3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the assisted-living / residential-care taxonomy that fits your facility — confirm the exact one ODHS wants for a community-based care contract. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

**4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

### 3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

#### What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Oregon branches — U.S. Bank, KeyBank, Umpqua/Columbia Bank, or a local option like OnPoint or Rivermark Community Credit Union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

### 3.5 Get your insurance — what you need, who sells it, and what to ask

A residential care facility houses and cares for vulnerable residents around the clock in a building you own or lease, so you need coverage built for a licensed senior-living setting — and your lender and ODHS application will expect proof:

| Coverage                          | What it protects against                                                                       | Must-have?                   |
|-----------------------------------|------------------------------------------------------------------------------------------------|------------------------------|
| General & Professional Liability  | Injury or property damage, plus claims about the CARE (personal care, medication, supervision) | Yes — foundational           |
| Abuse & Molestation               | Allegations of abuse of a resident — general liability EXCLUDES this                           | Yes — high exposure          |
| Property & Business Interruption  | Fire and damage to the building, plus lost income                                              | Yes                          |
| Directors & Officers / Management | Regulatory, employment, and governance claims                                                  | Recommended                  |
| Workers' Compensation             | Staff injuries in a 24-hour residential setting                                                | Required in OR for employees |
| Non-Owned & Hired Auto            | Staff driving residents to appointments or on outings                                          | Recommended                  |

**Match your professional liability to your care model.** A residential care facility administers medications and provides hands-on personal care, so confirm your professional-liability coverage reflects the actual care you deliver, not just a general-business policy.

#### Real places to get it

Buy through an independent agent or broker who writes SENIOR-LIVING / assisted-living programs — this is a specialty niche. Carriers and programs active in senior living include Philadelphia Insurance (PHLY), CNA, The Hartford, Great American, Markel, and senior-living specialty programs. Find a local Oregon agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for an RCF/ALF program. Confirm the carrier writes Oregon community-based care before you rely on a quote.

#### What to ask the insurance agent

*“Do you write Oregon residential care / assisted living facilities — which carriers, and what's the minimum premium?”*

*“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”*

*“Does professional liability cover our medication administration and personal care?”*

*“What limits does my lender and my ODHS contract expect?”*

*“Can you set up Oregon workers' comp for a 24-hour residential staff?”*

## PART 4

# New Construction & the Building

**In short:** *Because this is a facility, the building itself is part of the license — and Oregon reviews it before you open. If you're building, converting, or making major alterations, start with ODHS Facilities, Planning & Safety long before you plan to admit residents.*

- 1. Start with Facilities, Planning & Safety (FPS) — before the building permit.** Prospective RCF and ALF applicants must follow the new-construction process in OAR 411-054-0012 and work with ODHS FPS BEFORE you apply for a building permit.
- 2. Build to Oregon's specialty codes and Division 054.** Your facility must meet the Oregon Structural, Mechanical, Electrical, and Plumbing Specialty Codes, plus the RCF or ALF building requirements in Division 054.
- 3. Meet the ALF apartment rules exactly.** Each assisted living unit must have a lockable lever-handle entry door, a private bathroom, and a kitchenette — adaptable units are not acceptable — and a multi-floor building needs an elevator.
- 4. Finish before you license.** Send the Project Substantial Completion Notice to FPS about 30 days before your anticipated licensure so the building is ready for the survey.

*The building is the long pole in the tent. Construction, code review, and FPS sign-off take longer than any paperwork, so they set your real timeline. Engage FPS early, keep your architect and contractor working to the Division 054 requirements from the first drawings, and don't order furniture or set a move-in date until the building is on track — changing a building after the fact is far more expensive than getting the drawings right.*



## PART 5

# The Licensed Administrator

**In short:** Every Oregon RCF and ALF must employ a full-time administrator, on-site at least 40 hours a week, who holds an Oregon RCFA license. A facility cannot be licensed or operate without one — so secure yours before you apply.

Since January 1, 2022, the administrator must hold a Residential Care Facility Administrator (RCFA) license from the Oregon Health Authority, Health Licensing Office, under OAR Chapter 853 and ORS 678.710 to 678.820 — which requires meeting education and experience standards, completing the required administrator training, and passing the Oregon laws and rules examination. It's a real, ongoing license, maintained through annual continuing education and the standards of the Long Term Care Administrators Board.

**Secure yours early — and plan for coverage.** A facility cannot be licensed or operate without a qualified administrator, so line up your licensed administrator before you apply — whether that's you (if you become licensed) or someone you hire. If your administrator ever leaves, you must notify ODHS immediately and arrange a qualified interim administrator, so plan for coverage, not just for hiring.

Why Oregon treats this role so seriously: the administrator sets the tone for resident care, staffing, and compliance, and the state holds that person personally accountable through the license. For an owner, this has a practical consequence — either you invest the time to become a licensed administrator yourself, or you budget for a qualified one as a key hire and give them the authority to actually run the building. An absent or underqualified administrator is one of the fastest routes to a survey problem.

## PART 6

# Applying to ODHS

**In short:** Once your model, building, and administrator are lined up, you apply on the SE 0570 at least 60 days before you expect to be licensed. The application draws together your building, administrator, finances, policies, and staffing all at once.

## 6.1 File the SE 0570 — at least 60 days out

Submit your complete license application (SE 0570) at least sixty days before you expect to be licensed; if one entity owns the facility and another operates it, the operator files a management application (APD 0570M) too. Give yourself more than the minimum sixty days if you can — gaps in any one piece slow the whole thing down. Treat the sixty-day mark as the latest you should file, not the target.

## 6.2 What you submit

- **Ownership and fiscal responsibility:** each owner of 10% or more (5% if you serve Medicaid) is part of the application; you show proof of fiscal responsibility and a 12-month pro forma of revenues, expenditures, and resident days.
- **Your policies, disclosures, and agreements:** your policy and procedure manual, your written residency agreement, and your Uniform Disclosure Statement (and the memory care version if endorsed).
- **Your care and staffing systems:** your caregiver training protocol, your resident evaluation and service-planning tools, your staffing plan, and the name of the nurse who will oversee resident health services.
- **Background checks:** your Qualified Entity Designee (QED) is approved to submit background-check requests for owners, the administrator, and staff.
- **Administrator information:** the Administrator Reference Summary about 30 days before licensure.

*First-time operator converting a building? Budget for the six-month consultant. If you've never managed an RCF or ALF and you're converting an existing building, Oregon requires you to retain a consultant with residential-care experience for the first six months. Treat it as an asset, not just a cost — an experienced consultant keeps your first survey and your first months out of trouble.*

## PART 7

# Staffing, Training & Background Checks

**In short:** *Your residents are only as safe as your staff, and Oregon expects you to screen, train, and verify every caregiver — background checks through your QED before they provide care, and competency verified before they work independently.*

## 7.1 The staffing rules

- **Caregivers and universal workers:** direct-care staff — or universal workers who cross roles — provide personal care under your training program, matched to the services you offer.
- **Background checks first:** complete Oregon background checks through your QED BEFORE staff provide care.
- **Train and verify competency:** follow your caregiver training protocol and verify competency by observation and written or verbal testing before staff work independently — not just at hire, but on a schedule.
- **Dementia training for memory care:** if you're endorsed as a memory care community, staff complete approved dementia and person-centered dementia care training before providing care.
- **Enough staff, awake and available:** staff your facility to residents' actual needs around the clock — your staffing plan is part of your license.

## 7.2 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

## PART 8

# Residents — Admission, Rights & the Service Plan

**In short:** *How you admit and care for residents is the heart of the rules. You admit only residents whose needs you can meet, plan their care with them, and document it — and surveyors look hardest at that match and at how you respond when a resident's condition changes.*

## 8.1 Admission and the service plan

- **Disclosure and the residency agreement:** give every prospective resident your Uniform Disclosure Statement and a written residency agreement spelling out services, rates, and rights before move-in.
- **Evaluation at move-in:** evaluate each resident's needs and preferences at move-in, and only admit residents whose needs you're licensed and able to meet.
- **The person-centered service plan:** build a person-centered service plan for each resident; for a Medicaid resident, the APD or Area Agency on Aging case manager (the service-plan coordinator) leads it and you carry it out.
- **Resident rights:** honor residents' rights to dignity, choice, privacy, and to voice grievances without reprisal — and follow the rules on change of condition, health services, medications, and involuntary move-out.
- **Protect residents and report abuse:** prevent abuse, and report and investigate any suspected abuse under OAR 411-054-0028 and ORS 124; call 911 in an emergency.

## 8.2 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

## PART 9

# How You Get Paid — Private Pay & Oregon Medicaid

**In short:** Residential care and assisted living are paid two main ways: privately by residents and families, and by Oregon Medicaid (community-based care) for eligible residents. Most facilities serve a mix — and occupancy is the number that decides whether you thrive.

## 9.1 Private pay

Set clear rates in your residency agreement — a base rate for room and board plus the services each resident needs — and disclose them up front in your Uniform Disclosure Statement. Private pay needs no Medicaid contract, and rates in the Portland metro tend to run higher than in rural Oregon. Build your real costs — staffing around the clock, the licensed administrator, the nurse, food, insurance, and debt service on the building — into your rates.

## 9.2 Oregon Medicaid (community-based care)

To serve Medicaid residents, you contract with ODHS to provide community-based care. For a Medicaid resident, the APD or Area Agency on Aging case manager completes the person-centered service plan and authorizes services, and ODHS pays the facility under its rate structure while the resident pays their share toward room and board. Serving Medicaid opens a much larger population and steady referrals from case managers — the trade-off is contracting, the 5% ownership-disclosure threshold, and billing to the state's rules.

## 9.3 Build your pro forma on real numbers

Your 12-month pro forma is not just an application form — it's the plan that keeps you solvent. Base it on realistic occupancy that ramps up over months (not a full building on day one), your true staffing costs, and your actual private-pay and Medicaid mix.

- **Occupancy is the number:** a facility's revenue is beds filled times your rate, and most fixed costs — the building, the administrator, the nurse, minimum staffing — run whether a bed is full or empty. Occupancy is the whole game.
- **Plan a slow ramp and fund the gap:** new facilities most often get into trouble by assuming they'll fill quickly. Model a months-long lease-up and fund the gap with working capital.
- **Referrals drive occupancy:** relationships with hospital discharge planners, case managers, and senior-placement advisors take months to build — start that outreach the moment you're licensed, not when beds sit empty.
- **What this means for you:** model a few payer scenarios — mostly private, mostly Medicaid, and a blend — against your real costs before you commit, so you open with a clear-eyed view of how the facility actually makes money.

## PART 10

## What It Costs & How Long It Takes

**In short:** Residential care and assisted living are more capital-intensive than in-home care because you're licensing a building. Plan for the building (by far the largest cost), the ODHS license fee by bed count, the administrator and nurse, and a working-capital cushion for a slow lease-up.

### 10.1 Startup line items (typical — verify)

| Item                                 | What's in it                                                      | Ballpark                              |
|--------------------------------------|-------------------------------------------------------------------|---------------------------------------|
| The building                         | Construction, purchase, or conversion to Division 054 standards   | By far the largest cost — plan first  |
| ODHS license fee (by beds)           | New facility license, by licensed-bed tier                        | ~\$2,000 (1–15) up to ~\$6,000 (151+) |
| Memory care endorsement              | Added fee if endorsed                                             | ~\$50–\$100 (verify)                  |
| Licensed administrator               | A full-time RCFA on-site 40+ hrs/week                             | Core ongoing cost                     |
| Nursing & staffing                   | A nurse for health services + caregivers around the clock         | Your largest recurring expense        |
| Insurance & business setup           | Liability, property, workers' comp; SOS registration              | Get quotes — annual                   |
| Six-month consultant (if applicable) | Required for a first-time operator converting a building          | Budget for it                         |
| Working capital                      | Carry the building, administrator, nurse & staff through lease-up | Most-underestimated cost              |

### 10.2 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$5,500/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

### 10.3 Timeline & your first 90 days

|                                |                                                 |                                                                                                                              |
|--------------------------------|-------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------|
| <b>Before you apply</b>        | <b>Model, business, administrator, building</b> | Choose RCF/ALF/memory care; form the business; secure the RCFA; engage FPS on the building                                   |
| <b>Before you apply</b>        | <b>Policies &amp; systems</b>                   | Adopt policies, residency agreement, disclosures, training protocol & service-planning tools; line up your nurse & insurance |
| <b>Apply</b>                   | <b>SE 0570, 60 days out</b>                     | File with fiscal-responsibility proof & pro forma; set up background checks through your QED                                 |
| <b>Before licensure</b>        | <b>Finish &amp; survey</b>                      | Finish the building, send the Substantial Completion Notice, hire & train staff, pass the on-site survey                     |
| <b>After WRITTEN licensure</b> | <b>Open</b>                                     | Only now admit residents on a residency agreement & service plan; begin referral outreach                                    |

## PART 11

# Getting Your First Residents

**In short:** *Occupancy is a core operating discipline, not an afterthought. Residents come from the people families turn to — hospital discharge planners, APD and Area Agency on Aging case managers, and senior-placement advisors.*

## 11.1 Where your residents come from

- **Hospital discharge planners & social workers:** the top source of residents leaving a hospital stay who can't go home safely — make same-week admissions easy.
- **APD & Area Agency on Aging case managers:** they lead the service plan for Medicaid residents and route them to contracted facilities — being contracted and reliable keeps you in the rotation.
- **Skilled nursing facilities:** they discharge residents stepping down to a lower level of care.
- **Senior-placement advisors & home health/hospice:** placement services and in-home agencies refer families who now need a facility.

## 10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

## PART 12

## Who to Call & Exactly What to Ask — Every Verify, in One Place

**In short:** Your master contact and verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

### Licensing, the building & the administrator

| Who                                       | Contact          | What to verify / ask                                                                     |
|-------------------------------------------|------------------|------------------------------------------------------------------------------------------|
| ODHS APD — Community-Based Care Licensing | oregon.gov/odhs  | What a complete SE 0570 application requires; current fees & timeframes                  |
| ODHS Facilities, Planning & Safety (FPS)  | ODHS APD         | The new-construction process (OAR 411-054-0012); what to submit before a building permit |
| OHA Health Licensing Office — RCFA        | oregon.gov/oha   | How to qualify for and maintain the administrator license                                |
| Local fire marshal & building department  | your city/county | Oregon specialty codes; fire & life safety sign-off                                      |

### Business, insurance & billing IDs

| Who                                          | Contact                                 | What to verify / ask                                |
|----------------------------------------------|-----------------------------------------|-----------------------------------------------------|
| Oregon Secretary of State — Corporation Div. | sos.oregon.gov                          | Registering your entity (~\$100); the annual report |
| IRS                                          | irs.gov (EIN Assistant)                 | Getting your EIN (free)                             |
| NPPES / CMS                                  | nppes.cms.hhs.gov                       | Your Type 2 NPI (for the ODHS Medicaid contract)    |
| Insurance agent (senior-living program)      | trustedchoice.com; carriers in Part 3.5 | Liability + abuse + property + workers' comp        |

### Medicaid, background checks & referrals

| Who                                       | Contact                                                    | What to verify / ask                                                        |
|-------------------------------------------|------------------------------------------------------------|-----------------------------------------------------------------------------|
| Local APD office & Area Agency on Aging   | oregon.gov/odhs (find your AAA)                            | Contracting with ODHS to serve Medicaid; how case managers do service plans |
| ODHS Background Check Unit (via your QED) | ODHS APD                                                   | Setting up your QED and submitting background-check requests                |
| Adult Protective Services                 | report per OAR 411-054-0028 / ORS 124; 911 for emergencies | Reporting suspected abuse, neglect, or exploitation                         |

### Common questions, answered

#### Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

#### Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

#### Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

#### How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.



### **How long until it's profitable?**

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

### **Do I need experience in health care?**

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

### **What actually makes one agency win over another?**

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

### **Should I open an RCF or an ALF?**

It's a decision about the building and the resident experience: an RCF offers residential rooms and is the more flexible, lower-capital way to start; an ALF offers apartment-style units (lockable lever-handle door, private bathroom, kitchenette) and costs more to build. Many operators start with an RCF to learn to operate on a smaller scale and expand into apartments or memory care later. Build to the model you're licensing — apartments with kitchenettes are an ALF requirement.

### **Can I move a resident in while my license is pending?**

No — this is the single most serious mistake in Oregon. You may not move in even one resident, even briefly, until you have WRITTEN confirmation of licensure from the ODHS Community-Based Care Licensing Unit. Build your timeline around getting that written confirmation first.

### **Do I have to be the licensed administrator myself?**

No, but someone must be. Every RCF/ALF needs a full-time RCFA-licensed administrator on-site at least 40 hours a week. You can become licensed yourself or hire a qualified administrator as a key early hire — and you must line one up before you apply, plus plan for interim coverage if they ever leave.

### **Do I have to take Medicaid?**

No — you can run private-pay only, but serving Oregon Medicaid (community-based care) opens a much larger population and steady case-manager referrals. The trade-off is contracting with ODHS, the 5% ownership-disclosure threshold, and billing to the state's rules. Most facilities serve a private-pay/Medicaid mix; model a few payer scenarios before you commit.

## **Two worked examples**

### **Example 1 — A small residential care facility**

You start smaller with an RCF. You secure a licensed administrator, form the business, and work with FPS to bring a residential building up to the Division 054 residential-care requirements. You submit SE 0570 more than sixty days out with your policies, residency agreement, disclosures, fiscal-responsibility proof, and a 12-month pro forma built on a gradual lease-up. You run background checks through your QED, train your caregivers, pass the licensing survey, and receive written confirmation of licensure. Only then do you admit your first residents — on residency agreements and person-centered service plans — serving a mix of private-pay and Medicaid residents. Starting with an RCF keeps your building and capital requirements manageable while you learn to operate.

### **Example 2 — An assisted living facility with memory care**

You're building purpose-built apartments, so you license an ALF and add a memory care endorsement. You engage FPS before the building permit and build apartment units — each with a lockable lever-handle door, a private bathroom, and a kitchenette — plus a secure, purpose-built memory care area. You license your administrator, hire

and dementia-train your staff, and prepare your memory care disclosures. After the building is substantially complete and the survey is passed, you receive written licensure. You open the apartments and the memory care community, serving residents privately and, under your ODHS contract, through Oregon Medicaid — with the APD or AAA case manager leading each Medicaid resident's service plan.

### **You can do this**

Opening an Oregon residential care or assisted living facility is a real capital-and-compliance undertaking — a building, a licensed administrator, trained staff, and an ODHS survey — but it builds a durable asset and meets a deep, growing need. Choose the model that fits your capital (often an RCF to start), engage FPS on the building early, secure your RCFA administrator before you apply, build the operation around real staffing and person-centered service plans, and never move a resident in before written licensure. Verify each fee and contact as you go, and lean on the people in Part 12.

***Reminder.** Prepared for PolicyReady.org and grounded in OAR Chapter 411, Division 054 (and OAR 411-057 for memory care), ORS 443.400–443.455, and current Oregon Medicaid information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, rates, and contacts change — verify each with ODHS, the OHA Health Licensing Office, and qualified counsel before you rely on it. Confirm the current SE 0570 requirements, the license fees, and the RCFA licensing route before you rely on them.*